

Technical Case Study

Name: [INSERT] | Date: [INSERT]

Stage 1 | Figure Extraction

Assignment: extraction of data from figures

Design an approach to extracting specified numerical values from the figures and tables in the supplied set. Each value is to be returned with the method by which it was obtained and a confidence level. **A written outline is submitted ahead of the discussion; a working prototype is welcome but not required.**

Note: the figures are drawn from the public corporate and conference presentations of several development-stage oncology companies. A system built to ingest one name should not have seen these figures before.

SCOPE

The figure set

Six figures (see Appendix A), one of each type below. Four have had the slide's own summary values removed, so the quantity must be derived from the graphic rather than read from a caption.

Figure	What is to be read from it
Multi-arm Kaplan–Meier	The median for each arm, and the value for each arm at a specified landmark month, with the slide's summary table removed.
Waterfall of best percentage change	The proportion of patients in a specified dose subgroup beyond the 50 per cent reduction threshold, and the depth of a specified bar.
Log-scale pharmacokinetic or dose–response curve	A specified concentration at a specified time point, and the fold-multiple of that concentration over the marked threshold.
Forest plot of subgroups	The hazard ratio and the confidence interval for two specified rows.
Table delivered only as a raster image	A specified efficacy and safety value for two named trials, and the patient number each is drawn from.
Spider plot converted to a survival curve	The progression-free survival curve implied by the per-patient trajectories: the median, the value at a specified landmark, and the range the figure cannot resolve.

OUTPUT

Deliverables

- 1 The approach.** No more than two pages: how the problem would be attacked, where the approach is expected to fail, and what would be built first. Include an estimate of what proportion could run unattended at high confidence, what would require analyst review, and the accuracy ceiling you would expect.
- 2 The extractions.** Your reading of each requested quantity, with a confidence level attached to each.
- 3 The evaluation.** How it would be established that the system works, and what would be measured to know when it does not.
- 4 The prototype, if any.** Optional. Any working code produced, executable as submitted, with the approximate cost of one run.

ASSESSMENT AND ADMINISTRATION

Basis of assessment and administration

Judgement will be weighted above coverage, and a candid account of where an approach fails carries more weight than one identifying none.

Tooling. Unrestricted as to language, framework, and model. State what was used.

Data. Public sources only. No paid subscriptions, and no collection method you would be uncomfortable describing.

Format. The figure set and this brief are issued 48 hours ahead of a 45 to 60 minute discussion in which the approach is walked through and pressure-tested.

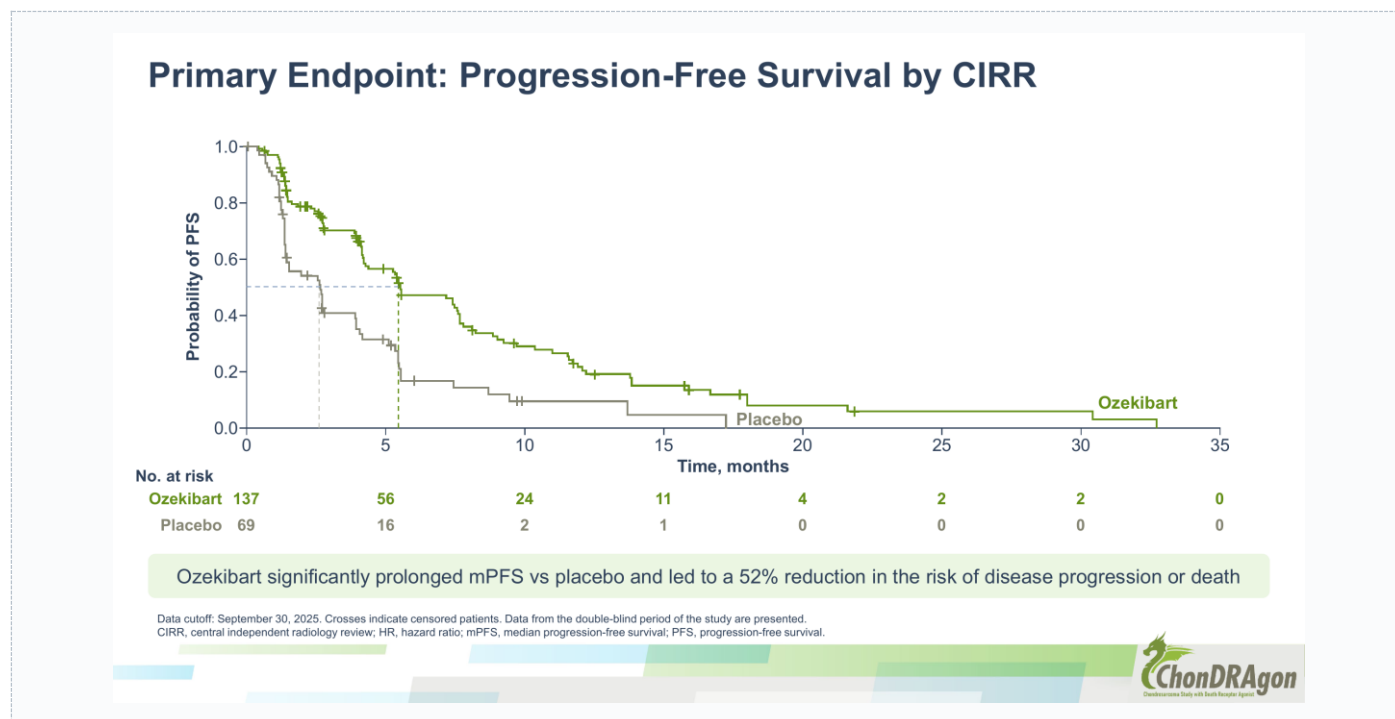
Submission. Return the written approach, your extractions, the evaluation and prototype (optional) to marcel@perceptivelife.com at least 5 hours before the in-person discussion.

APPENDIX

A The figure set

The six figures referenced in section 2.2 follow. Each is supplied as a flattened raster image with no underlying text layer. The quantities to be returned are listed beneath each figure; every one is to carry the method by which it was obtained and a confidence level.

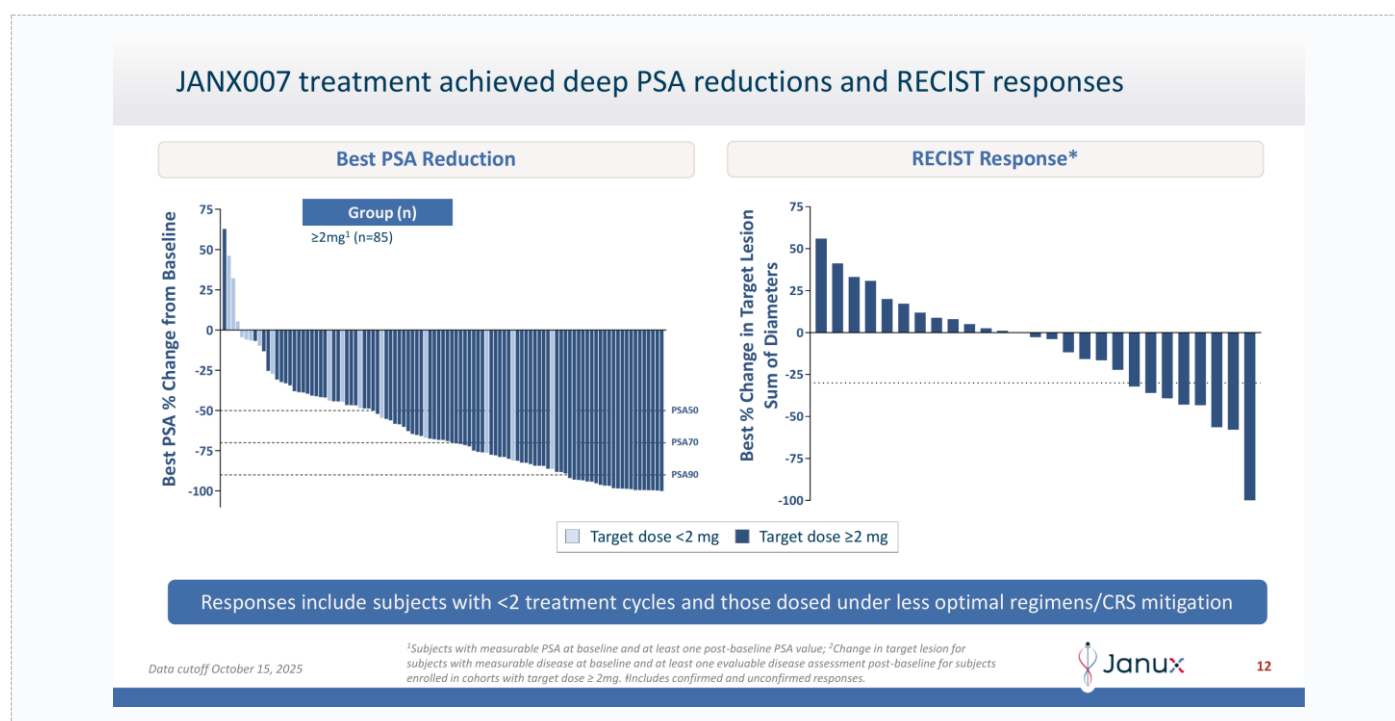
Figure 1 – Multi-arm Kaplan-Meier



Source: Inhibrx Biosciences, ChonDRAGON primary data (ozekibart in chondrosarcoma), BSG oral presentation, February 2026, slide 8. The summary table has been removed from the original slide; the median and landmark values are to be read from the plotted curve.

Values to return: (a) median progression-free survival for each arm; (b) the PFS probability for each arm at 6 months. The slide's summary table has been removed, so both are to be read from the plotted curve; state the method used for each and the confidence attached to it.

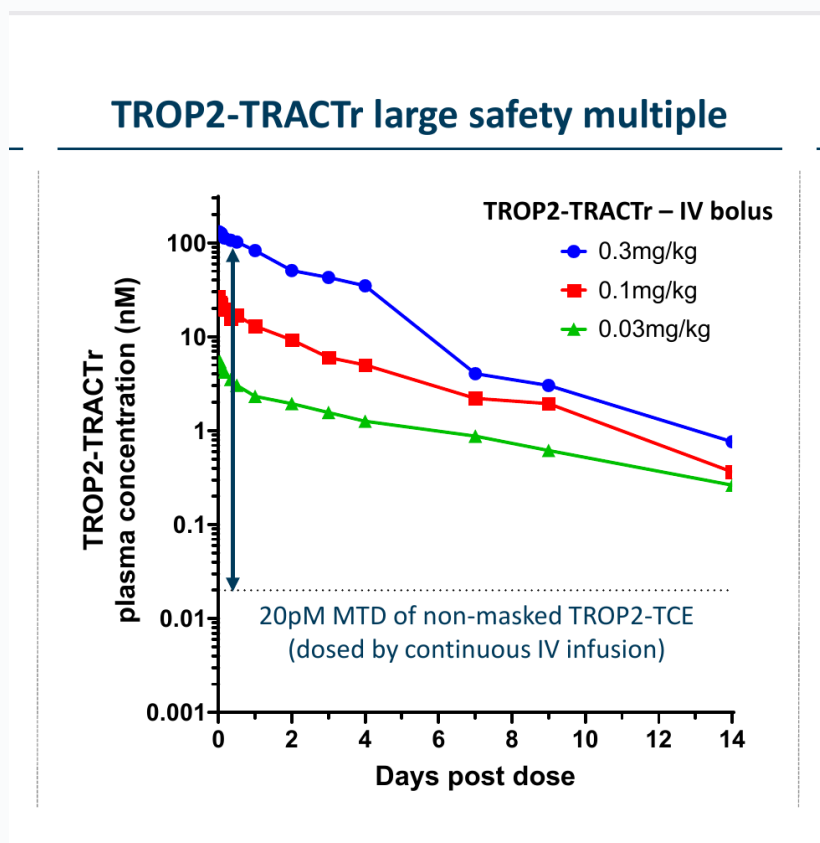
Figure 2 – Waterfall of best percentage change



Source: Janux Therapeutics, JANX007 programme update deck, 1 December 2025, slide 12. The PSA50, PSA90 and ORR summary values have been removed from the original slide; the proportions are to be derived from the plotted bars.

Values to return: (a) the proportion of the ≥ 2 mg subgroup with a PSA reduction of at least 50 per cent, and of at least 90 per cent; (b) the depth of the deepest bar, and the height of the leftmost bar; (c) in the right-hand panel, the number of patients plotted and the proportion of them beyond the -30 per cent line. State the denominator each proportion is taken over.

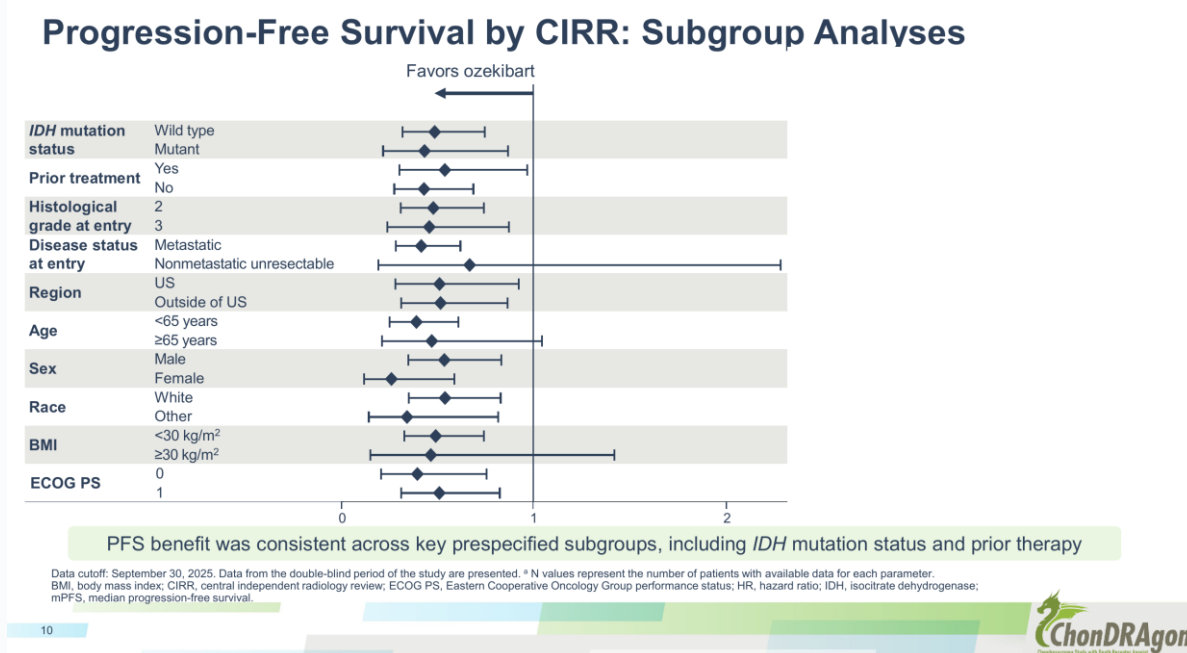
Figure 3 – Log-scale pharmacokinetic or dose–response curve



Source: Janux Therapeutics, R&D Day, July 2025, slide 23 (centre panel). The stated safety multiple has been removed from the original slide; it is to be derived from the plotted curve and the marked threshold.

Values to return: (a) the plasma concentration at day 14 for the 0.03 mg/kg dose level, in nM; (b) the concentration of the marked non-masked TCE maximum tolerated dose threshold, in nM; (c) the fold-multiple of the 0.3 mg/kg peak concentration over that threshold; (d) the two sampled timepoints between which the 0.03 mg/kg curve crosses 1 nM. Note where the axis carries no minor gridlines and how that bears on confidence.

Figure 4 – Forest plot of subgroups



Source: Inhibrx Biosciences, ChonDRAGON primary data, BSG oral presentation, February 2026, slide 10. The numerical columns of the original slide have been removed; the values are to be read from the graphic.

Values to return: (a) the hazard ratio and 95 per cent confidence interval for IDH wild type, and for ECOG PS 0; (b) every subgroup whose confidence interval crosses 1. The numerical columns have been removed from this slide, so each value is to be read from the graphic.

Figure 5 – Table delivered only as a raster image

CIS with or without Ta/T1					
Trial (Status)	BOND-003 COHORT C (Ph3 Ongoing)	SunRISe-1 (Approved)	QUILT 3.032 (Approved)	NCT02773849 (Approved)	KEYNOTE-057 (Approved)
Drug	Cretostimogene	TAR-200	N-803 + BCG	Nadofaragene	Pembrolizumab
Mechanism	Oncolytic Immunotherapy	Gemcitabine via In-Dwelling Device	IL-15 Superagonist + BCG combo	Gene Therapy Secreting IFN	Checkpoint Inhibitor
RoA	Intravesical	Transurethral Procedure	Intravesical	Intravesical	Intravenous
Efficacy Population	110	83	77	98	96
CR at Any Time	75.5% (83/110) [95% CI: 66% - 83%]	82.4% (70/83) [95% CI: 73% - 90%]	62.3% (48/77) [95% CI: 51% - 73%]	51.0% (50/98) [§] [95% CI: 41% - 61%]	40.6% (39/96) [95% CI: 31% - 51%]
CR at 12 Mo	46.4% (51/110) [95% CI: 37% - 56%]	K-M: 45.9% (39/83)	36.4% (28/77) [‡]	24.3% (25/103)	18.8% (18/96)
CR at 24 Mo	41.8% (46/110) [‡] [95% CI: 33% - 52%]	Not Reported	24.7% (19/77) [‡]	19.4% (20/103)	9.4% (9/96) [‡]
12M DOR	K-M: 64.2% [95% CI: 52% - 74%]	Observed: 52.9% K-M: 56.2%	58%	46%	46%
24M DOR	K-M: 60.1% [95% CI: 48% - 70%]	K-M: 51.8%	40%	Not Reported	Not Reported
Free from Progression to MIBC	96.4% at 24 month	94.3%	90%	94%	89%
Cystectomy Free Survival	89.2% at 12 month 81.3% at 24 month [‡]	86.6% at 12 month [‡]	59% [‡]	64% at 24-month	84%
Median Time to AE Resolution	1 Day	3.1 weeks	Not Reported	Not Reported	Not Reported
Grade 3+ TRAE	0%	13%	Not reported; 16% SAE	4%	13%
TR discontinuation	0%	3.5%	7%	3%	11%

[†] These data are based on head-to-head or comparator studies. Differences exist between study designs and subject characteristics, and caution should be exercised when comparing data across studies. From published data. A AB 9 ongoing CRs remained in response at the 24-month assessment, as well as 3 additional patients confirmed to be in CR at Month 24 after further follow-up and central pathology adjudication. *CR = complete response. †CR = complete response. ‡Derived from ADJUTANAL plus BCG Package Insert Page 22 and using DOR at 12 months and DOR at 24 months to estimate 12 months and 24 months landmark CR rates. ADJUTANAL Package Insert December 2020 and Summary Basis for Regulatory Action. § Derived from QUARCAD 2021. Baker et al presentation DOR at 24 months to estimate 24 months landmark CR rate. †† Derived from KEYNOTE-057 presentation slide. Keynote 057 presentation slide for pembrolizumab (T) presented. Baker, et al., J Clin Oncol. 2020;38(15):1651-1661. [abstract]. Available at: <https://www.asco.org/journal/jco/abstract/38/15/1651>. Accessed April 2024.

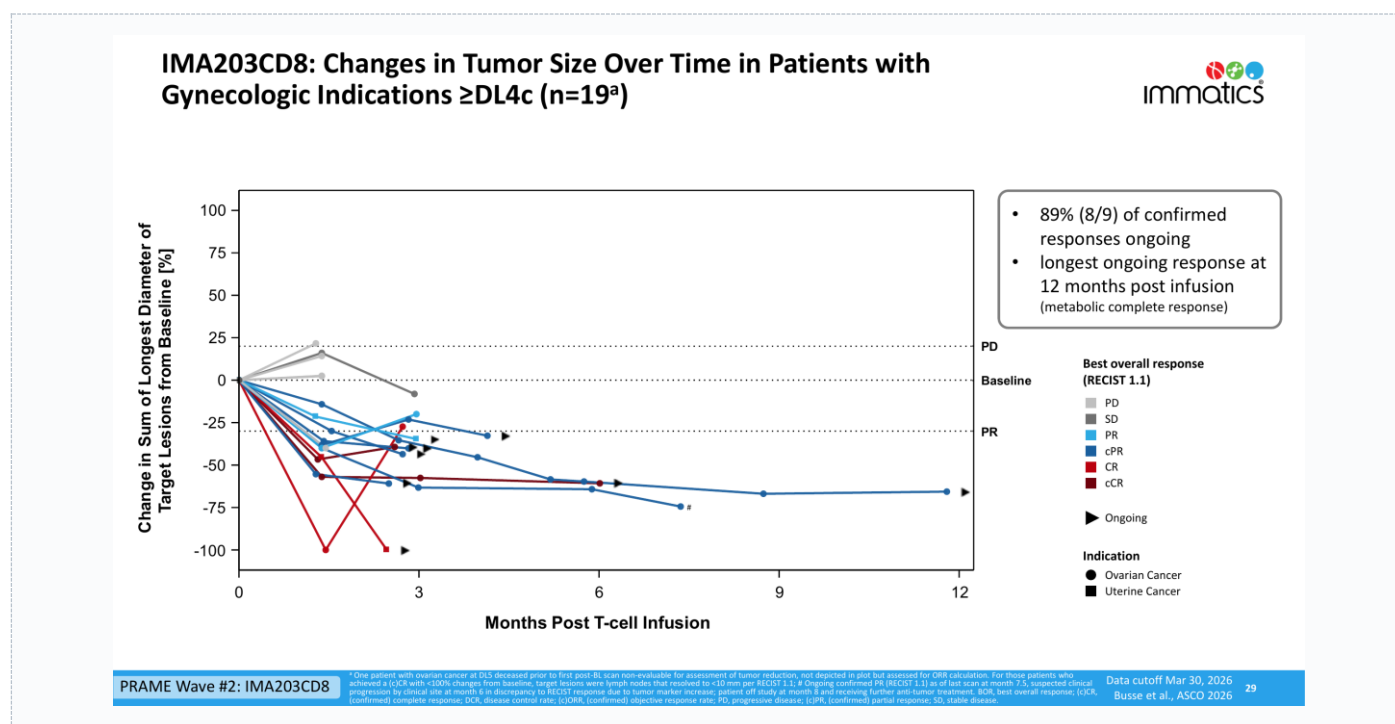
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Source: CG Oncology investor presentation, August 2026, slide 13.

Values to return: (a) complete response at 12 months for BOND-003 Cohort C and for SunRISe-1, each with the patient number it is drawn from; (b) the 24-month duration of response for QUILT 3.032; (c) the grade 3+ treatment-related adverse event rate for TAR-200; (d)

for NCT02773849, why the patient number differs between complete response at any time and complete response at 12 months. Where a cell reports more than one estimator, return both and say which is comparable across columns.

Figure 6 – Spider plot converted to a progression-free survival curve



Source: Immatics Corporate Presentation, August 2026, slide 29 (IMA203CD8, gynaecologic indications at doses $\geq$ DL4c, n=19).

Values to return: (a) the progression-free survival curve implied by this figure for the cohort shown, presented as a table of month and surviving proportion; (b) the implied median PFS; (c) the implied PFS at 6 months; (d) the rule you applied to convert each trajectory into an event or a censoring time, and how that rule treats a trajectory ending in an ongoing marker and a trajectory whose only post-baseline assessment sits marginally above baseline; (e) the range of medians consistent with the figure, and what the figure does not carry that drives that range.